

Access guide — cca-mets-scope-unknown-z3p9

How to access each therapy: trial contacts + manufacturer medical-info

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How a patient or treating team could practically access each intervention in this case's dossier. Trial recruitment contacts and manufacturer medical-information lines are captured for direct outreach. The number in the first column of the summary table is the entry's identifier in the per-intervention sections below — use it to find the deep section quickly. Information ages — each row carries a [Verified](#) date; re-screen before relying on a specific contact or trial slot.

Summary

#	Intervention	Modality	Access status	Regulatory	Recommended first action
1	Dabrafenib + trametinib	small_molecule	Standard of care + Off-label use	FDA-approved (June 2022) tumor-agnostic for unresectable or metastatic BRAF V600E solid tumors after prior therapy; also approved in melanoma, NSCLC, thyroid, and low-grade glioma	Confirm BRAF V600E by NGS or PCR (VE1 IHC alone is not sufficient).
2	Entrectinib	small_molecule	Standard of care + Off-label use	FDA-approved (accelerated, August 2019) tumor-agnostic for NTRK fusion-positive solid tumors; also approved in ROS1-positive NSCLC	Run an RNA-based fusion panel (or pan-TRK IHC as a screen) and confirm an NTRK1/2/3 fusion.
3	Futibatinib	small_molecule	Standard of care + Off-label use	FDA-approved (accelerated, September 2022) for previously treated, unresectable locally advanced or metastatic intrahepatic cholangiocarcinoma with an FGFR2 fusion or rearrangement; also EMA-approved	Confirm an FGFR2 fusion or rearrangement and document intrahepatic subsite, since the label is intrahepatic-specific.

4	Ivosidenib	small_molecule	Standard of care + Off-label use	FDA-approved (August 2021) for previously treated, locally advanced or metastatic cholangiocarcinoma with a susceptible IDH1 mutation; also approved in IDH1-mutant AML and EMA-approved	Order tumor DNA NGS covering the IDH1 R132 hotspot and confirm a susceptible mutation.
5	Larotrectinib	small_molecule	Standard of care + Off-label use + Clinical trial	FDA-approved (November 2018, full approval 2021) tumor-agnostic for NTRK fusion-positive solid tumors	Confirm an NTRK1/2/3 fusion on an RNA-based fusion panel.
6	Pembrolizumab	monoclonal_antibody	Standard of care + Off-label use + Clinical trial	FDA-approved tumor-agnostic for MSI-H/dMMR solid tumors (full approval) and for TMB-high (≥ 10 mut/Mb) solid tumors (accelerated); broadly approved across many indications	Run MMR IHC plus reflex MSI assay and confirm TMB on the NGS panel.
7	Pemigatinib	small_molecule	Standard of care + Off-label use	FDA-approved (accelerated, April 2020) for previously treated, unresectable locally advanced or metastatic cholangiocarcinoma with an FGFR2 fusion or rearrangement detected by an FDA-approved test; also EMA-approved	Order RNA-based fusion NGS (or DNA NGS with fusion calling) and confirm an FGFR2 fusion or rearrangement on an FDA-approved test.
8	Selpercatinib	small_molecule	Standard of care + Off-label use + Clinical trial	FDA-approved (accelerated, September 2022) tumor-agnostic for RET fusion-positive solid tumors; also approved in RET fusion NSCLC and RET-mutant/fusion thyroid cancer	Confirm a RET fusion on an RNA-based fusion panel.

9	Zanidatamab	bispecific_other	Standard of care + Off-label use + Clinical trial	FDA-approved (accelerated, November 2024) for previously treated, unresectable or metastatic HER2-positive (IHC 3+) biliary tract cancer	Run HER2 IHC with reflex ISH (and NGS) and confirm IHC 3+ for the on-label path.
10	Adagrasib	small_molecule	Off-label use + Clinical trial	FDA-approved (accelerated) for KRAS G12C-mutated NSCLC (monotherapy) and for KRAS G12C-mutated colorectal cancer (with cetuximab); no biliary-tract indication	Confirm KRAS G12C specifically on tumor DNA NGS; other KRAS variants do not qualify for a G12C inhibitor.
11	Trastuzumab deruxtecan (T-DXd)	ADC	Off-label use + Clinical trial	FDA-approved (accelerated, April 2024) tumor-agnostic for previously treated, unresectable or metastatic HER2-positive (IHC 3+) solid tumors; biliary tract cancer was part of the supporting data	Run HER2 IHC with reflex ISH and document the exact IHC score, since 3+ is the on-label threshold and drives expected benefit.
12	TYRA-200	small_molecule	Clinical trial	Investigational; no regulatory approval anywhere	Confirm an activating FGFR2 alteration first, since eligibility turns on it.

Standard of care (9)

1. Dabrafenib + trametinib

Aliases: dabrafenib, GSK2118436, Tafinlar, trametinib, GSK1120212, Mekinist

Modality: small_molecule · **Regulatory:** FDA-approved (June 2022) tumor-agnostic for unresectable or metastatic BRAF V600E solid tumors after prior therapy; also approved in melanoma, NSCLC, thyroid, and low-grade glioma · **Guidelines:** NCCN-listed for BRAF V600E biliary tract cancer under the tumor-agnostic indication · **Geographic scope:** US (and broadly available); approved · **Verified:** 2026-06-01

A confirmed BRAF V600E opens this combination through two routes that land in the same place: the

tumor-agnostic label and the ROAR biliary precedent, both prescribable without a trial. The gate is a molecularly confirmed V600E; a VE1 IHC screen is suggestive but needs NGS or PCR confirmation, and non-V600E BRAF variants do not qualify.

Next steps

1. Confirm BRAF V600E by NGS or PCR (VE1 IHC alone is not sufficient).
2. If positive, the oncologist prescribes the combination on-label; call Novartis at 1-888-669-6682 for dosing or interaction questions.
3. Counsel on pyrexia, the characteristic combination class effect, and plan management.
4. Confirm the alteration is V600E specifically, since class 2/3 BRAF variants are not covered.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02024110		completed	unconfirmed	—

Manufacturer / sponsor contact

- **Company:** Novartis
- **Med info phone:** 1-888-669-6682
- **Product info:** <https://www.us.tafinlarmekinist.com/>
- **Notes:** 1-888-669-6682 is the Novartis US medical-information / customer-interaction line. No public medical-info email found. Both drugs are dosed together as the approved combination.

Payer / coverage: The tumor-agnostic BRAF V600E label makes biliary use on-label, so coverage is generally straightforward once a confirmed V600E is documented. Non-V600E class 2/3 BRAF alterations do not qualify.

2. Entrectinib

Aliases: entrectinib, RXDX-101, Rozlytrek

Modality: small_molecule · **Regulatory:** FDA-approved (accelerated, August 2019) tumor-agnostic for NTRK fusion-positive solid tumors; also approved in ROS1-positive NSCLC · **Guidelines:** NCCN-listed for NTRK fusion-positive solid tumors under the tumor-agnostic indication · **Geographic scope:** US (and broadly available); approved · **Verified:** 2026-06-01

If RNA-based fusion NGS finds an NTRK1/2/3 fusion, entrectinib is on-label tumor-agnostic therapy the oncologist prescribes directly. NTRK fusions are rare in biliary cancer (under 1%), so the practical question is whether the fusion panel catches one; entrectinib's CNS penetration is a point in its favor if there is brain disease.

Next steps

1. Run an RNA-based fusion panel (or pan-TRK IHC as a screen) and confirm an NTRK1/2/3 fusion.
2. If positive, weigh entrectinib against larotrectinib; entrectinib has CNS activity, larotrectinib has more durable response data.
3. Prescribe on-label and enroll in Genentech Access Solutions; call 1-800-821-8590 for medical-information

questions.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02528267		completed	unconfirmed	—

Manufacturer / sponsor contact

- **Company:** Genentech / Roche
- **Med info phone:** 1-800-821-8590
- **Product info:** <https://www.gene.com/medical-professionals/medicines/rozlytrek>
- **Notes:** 1-800-821-8590 is the Genentech Medical Communications line. Genentech Access Solutions supports benefits and copay. No public medical-info email found.

Payer / coverage: On-label coverage requires a confirmed NTRK1/2/3 fusion. Genentech Access Solutions handles benefits verification and copay assistance. CNS-penetrant, which can matter if brain involvement is present.

3. Futibatinib

Aliases: futibatinib, TAS-120, Lytgobi

Modality: small_molecule · **Regulatory:** FDA-approved (accelerated, September 2022) for previously treated, unresectable locally advanced or metastatic intrahepatic cholangiocarcinoma with an FGFR2 fusion or rearrangement; also EMA-approved · **Guidelines:** NCCN-listed for FGFR2 fusion/rearrangement-positive intrahepatic biliary tract cancer, 2L+ · **Geographic scope:** US (and EU); approved and commercially available · **Verified:** 2026-06-01

Futibatinib is the other approved FGFR inhibitor, and it holds activity against some kinase-domain mutations that drive resistance to pemigatinib, which makes it useful for sequencing rather than just as an alternative first FGFR drug. Same gate as pemigatinib: a confirmed FGFR2 fusion or rearrangement, and the on-label indication is specific to intrahepatic disease.

Next steps

1. Confirm an FGFR2 fusion or rearrangement and document intrahepatic subsite, since the label is intrahepatic-specific.
2. If positive, prescribe directly and enroll in Taiho Oncology Patient Support at 1-844-824-4648.
3. Consider futibatinib specifically in the post-pemigatinib setting given its activity against some acquired resistance mutations.
4. Monitor phosphate and schedule ophthalmologic exams per the FGFR class profile.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
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Manufacturer / sponsor contact

- **Company:** Taiho Oncology
- **Med info phone:** 1-844-824-4648
- **Product info:** <https://www.lytgobi.com/>
- **Notes:** 1-844-TAIHO-4U (Mon-Fri 8am-8pm ET) routes to Taiho Oncology Patient Support for benefits review, copay assistance, and specialty-pharmacy coordination. No public medical-info email found.

Payer / coverage: On-label coverage for an FGFR2-positive intrahepatic result is routine; Taiho Oncology Patient Support handles benefits and copay. The label is specific to intrahepatic CCA, so subsite documentation matters for reimbursement.

4. Ivosidenib

Aliases: ivosidenib, AG-120, Tibsovo

Modality: small_molecule · **Regulatory:** FDA-approved (August 2021) for previously treated, locally advanced or metastatic cholangiocarcinoma with a susceptible IDH1 mutation; also approved in IDH1-mutant AML and EMA-approved · **Guidelines:** NCCN-listed for IDH1-mutant biliary tract cancer, 2L+ · **Geographic scope:** US (and EU); approved and commercially available · **Verified:** 2026-06-01

If NGS returns an IDH1 R132 mutation, ivosidenib is approved second-line therapy the oncologist can prescribe directly. ClarIDHy was positive on PFS (HR 0.37) but the absolute PFS benefit is modest and the OS signal was blunted by heavy crossover, so this is a real option with honest limits, contingent on the hotspot testing positive.

Next steps

1. Order tumor DNA NGS covering the IDH1 R132 hotspot and confirm a susceptible mutation.
2. If positive, prescribe directly and enroll in ServierONE at 1-800-813-5905 for benefits and copay support.
3. Establish baseline ECG and monitor for QT prolongation on treatment.
4. Call the same line for medical-information questions; report adverse events to 1-800-807-6124.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02939857		completed	unconfirmed	—

Manufacturer / sponsor contact

- **Company:** Servier (formerly Agios)
- **Med info phone:** 1-800-813-5905
- **Product info:** <https://www.tibsovo.com/cca>
- **Notes:** 1-800-813-5905 (Mon-Fri 8am-8pm ET) is the ServierONE patient-support line. Adverse-event reporting is 1-800-807-6124. No public medical-info email found.

Payer / coverage: On-label coverage for a confirmed IDH1 R132 result is routine; ServierONE handles benefits review and copay assistance. ECG monitoring for QT prolongation is part of safe use.

5. Larotrectinib

Aliases: larotrectinib, LOXO-101, Vitrakvi

Modality: small_molecule · **Regulatory:** FDA-approved (November 2018, full approval 2021) tumor-agnostic for NTRK fusion-positive solid tumors · **Guidelines:** NCCN-listed for NTRK fusion-positive solid tumors under the tumor-agnostic indication · **Geographic scope:** US (and broadly available); approved · **Verified:** 2026-06-01

Same NTRK-fusion gate as entrectinib: confirm a fusion and larotrectinib is on-label tumor-agnostic therapy, prescribable without a trial. Larotrectinib is the more selective TRK inhibitor with the most durable responses in the pooled data, so it is a reasonable default when there is no CNS disease driving the choice.

Next steps

1. Confirm an NTRK1/2/3 fusion on an RNA-based fusion panel.
2. If positive, choose between larotrectinib (more durable responses) and entrectinib (CNS activity) based on disease distribution.
3. Prescribe on-label and enroll in Access Services by Bayer at 1-800-288-8374; call 1-888-842-2937 for product questions.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02526431		completed	unconfirmed	—

Manufacturer / sponsor contact

- **Company:** Bayer (with Loxo Oncology)
- **Med info phone:** 1-888-842-2937
- **Product info:** <https://www.vitrakvi-us.com/>
- **Notes:** 1-888-842-2937 is the Vitrakvi information line. Access Services by Bayer (1-800-288-8374, Mon-Fri 8am-8pm ET) provides a nurse/pharmacist line and benefits support. No public medical-info email found.

Payer / coverage: On-label coverage requires a confirmed NTRK1/2/3 fusion. Access Services by Bayer handles benefits review and copay. Among the fusion inhibitors, larotrectinib reported the most durable responses (median DoR about 35 months).

6. Pembrolizumab

Aliases: pembrolizumab, MK-3475, Keytruda, lambrolizumab

Modality: monoclonal_antibody · **Regulatory:** FDA-approved tumor-agnostic for MSI-H/dMMR solid tumors (full

approval) and for TMB-high (≥ 10 mut/Mb) solid tumors (accelerated); broadly approved across many indications · **Guidelines:** NCCN-listed for MSI-H/dMMR and TMB-high biliary tract cancer under the tumor-agnostic indications · **Geographic scope:** US (and broadly available); approved · **Verified:** 2026-06-01

If MMR IHC, an MSI assay, or NGS-derived TMB comes back MSI-H/dMMR or TMB-high, pembrolizumab is on-label tumor-agnostic therapy the oncologist can prescribe directly. Both markers are uncommon in biliary cancer (MSI-H roughly 1-3%) but decision-changing when present, and the same comprehensive panel that resolves the other axes returns the TMB value.

Next steps

1. Run MMR IHC plus reflex MSI assay and confirm TMB on the NGS panel.
2. If MSI-H/dMMR or TMB-high, prescribe on-label; the MSI-H/dMMR path is the more robust full approval.
3. Enroll in the Merck Access Program for prior authorization and copay support; call 1-800-672-6372 for medical-information questions.
4. Counsel on immune-mediated adverse events as the characteristic checkpoint-inhibitor risk.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02628067		active not recruiting	unconfirmed	—

Manufacturer / sponsor contact

- **Company:** Merck
- **Med info phone:** 1-800-672-6372
- **Product info:** <https://www.keytruda.com/>
- **Notes:** 1-800-672-6372 is the Merck National Service Center / medical-information line. The Merck Access Program supports benefits and copay. No public medical-info email found.

Payer / coverage: On-label coverage requires an FDA-approved MSI-H/dMMR or TMB-high test result. MSI-H/dMMR is the cleaner full-approval path; the TMB-high indication is accelerated and occasionally draws more payer scrutiny. The Merck Access Program assists with prior authorization and copay.

7. Pemigatinib

Aliases: pemigatinib, INCB054828, Pemazyre

Modality: small_molecule · **Regulatory:** FDA-approved (accelerated, April 2020) for previously treated, unresectable locally advanced or metastatic cholangiocarcinoma with an FGFR2 fusion or rearrangement detected by an FDA-approved test; also EMA-approved · **Guidelines:** NCCN-listed for FGFR2 fusion/rearrangement-positive biliary tract cancer, 2L+ · **Geographic scope:** US (and EU); approved and commercially available · **Verified:** 2026-06-01

If comprehensive profiling returns an FGFR2 fusion or rearrangement, pemigatinib becomes standard second-line therapy a treating oncologist can prescribe directly, no trial needed. The whole path hinges on that one positive test, which is exactly the rank-1 workup this case is waiting on.

Next steps

1. Order RNA-based fusion NGS (or DNA NGS with fusion calling) and confirm an FGFR2 fusion or rearrangement on an FDA-approved test.
2. If positive, the treating oncologist writes a standard prescription and enrolls the patient in IncyteCARES at 1-855-452-5234 for benefits and copay support.
3. Plan baseline and serial ophthalmologic exams and phosphate monitoring before starting.
4. Call Incyte Medical Information at 1-855-463-3463 for any dosing or drug-interaction questions.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02924376		completed	unconfirmed	—

Manufacturer / sponsor contact

- **Company:** Incyte
- **Med info phone:** 1-855-463-3463
- **Med info email:** medinfo@incyte.com
- **Product info:** <https://www.pemazyre.com/>
- **Notes:** IncyteCARES Oncology patient-assistance line is 1-855-452-5234 (Mon-Fri 8am-8pm ET) for benefits review and copay help.

Payer / coverage: On-label use for an FGFR2-positive result is routinely covered; an FDA-approved companion test result is part of the label. IncyteCARES handles benefits review and copay assistance. If the FGFR2 test is negative, this drug is not indicated.

8. Selpercatinib

Aliases: selpercatinib, LOXO-292, Retevmo

Modality: small_molecule · **Regulatory:** FDA-approved (accelerated, September 2022) tumor-agnostic for RET fusion-positive solid tumors; also approved in RET fusion NSCLC and RET-mutant/fusion thyroid cancer ·

Guidelines: NCCN-listed for RET fusion-positive solid tumors under the tumor-agnostic indication · **Geographic scope:** US (and broadly available); approved · **Verified:** 2026-06-01

If RNA-based fusion NGS finds a RET fusion, selpercatinib is on-label tumor-agnostic therapy the oncologist prescribes directly. RET fusions are rare in biliary cancer, so the practical question is whether the fusion panel catches one; the LIBRETTO-001 basket explicitly covered non-lung, non-thyroid tumors.

Next steps

1. Confirm a RET fusion on an RNA-based fusion panel.
2. If positive, prescribe on-label and enroll in the Lilly Oncology Support Center.
3. Call Lilly medical information at 1-800-545-5979 for dosing or interaction questions.
4. Monitor blood pressure and transaminases as the characteristic RET-inhibitor class effects.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03157228		active not recruiting	unconfirmed	1-877-285-4559

Manufacturer / sponsor contact

- **Company:** Eli Lilly (with Loxo Oncology)
- **Med info phone:** 1-800-545-5979
- **Product info:** <https://www.retevmo.com/>
- **Notes:** 1-800-545-5979 is the Lilly Answers Center / medical-information line. The Lilly study line in the registry is 1-877-285-4559. Lilly Oncology Support Center assists with benefits and copay. No public medical-info email found.

Payer / coverage: On-label coverage requires a confirmed RET fusion. Lilly Oncology Support Center handles benefits verification and copay. Hypertension and transaminase elevation are the characteristic class effects to monitor.

9. Zanidatamab

Aliases: zanidatamab, ZW25, zanidatamab-hrii, Ziihera

Modality: bispecific_other · **Regulatory:** FDA-approved (accelerated, November 2024) for previously treated, unresectable or metastatic HER2-positive (IHC 3+) biliary tract cancer · **Guidelines:** NCCN-listed for HER2-positive (IHC 3+) biliary tract cancer, 2L+ · **Geographic scope:** US; first-line trial NCT06282575 recruiting at multiple US sites · **Verified:** 2026-06-01

For a treatment-naive patient who tests HER2 IHC 3+, the choice is between on-label second-line zanidatamab (approved on the HERIZON-BTC-01 data) and the first-line randomized trial NCT06282575, which is recruiting across US sites. The load-bearing test is HER2 IHC with reflex ISH, scored on the companion antibody rather than read off an NGS copy-number call.

Next steps

1. Run HER2 IHC with reflex ISH (and NGS) and confirm IHC 3+ for the on-label path.
2. If treatment-naive and HER2-positive, contact the NCT06282575 central line at 215-832-3750 to check first-line trial slots before committing to second-line therapy.
3. For approved second-line use, prescribe directly and enroll in JazzCares via ziiherahcp.com.
4. Baseline and serial cardiac (LVEF) monitoring before and during treatment.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
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NCT06232575	recruiting	unconfirmed	Clinical Trial Disclosure & Transparency; 215-832-3750
NCT04426891	active not recruiting	unconfirmed	—

Manufacturer / sponsor contact

- **Company:** Jazz Pharmaceuticals (with BeiGene)
- **Product info:** <https://www.ziihera.com/>
- **Notes:** JazzCares is the patient-support program (ziihera.com / ziiherahcp.com). No standalone medical-info phone or compassionate-use email surfaced in public sources; the registry trial-disclosure line is 215-832-3750.

Payer / coverage: On-label coverage requires HER2 IHC 3+; the gating test is HER2 IHC with reflex ISH, not NGS copy number alone. JazzCares handles access support. Cardiac monitoring is warranted given the ejection-fraction signal.

Off-label use (2)

10. Adagrasib

Aliases: adagrasib, MRTX849, Krazati

Modality: small_molecule · **Regulatory:** FDA-approved (accelerated) for KRAS G12C-mutated NSCLC (monotherapy) and for KRAS G12C-mutated colorectal cancer (with cetuximab); no biliary-tract indication · **Guidelines:** No biliary-specific guideline listing; off-label for biliary tract cancer · **Geographic scope:** US; off-label for biliary tract cancer · **Verified:** 2026-06-01

Adagrasib is approved for KRAS G12C NSCLC and colorectal cancer but not for biliary cancer, so a biliary patient with a confirmed G12C would access it off-label, supported by the small published KRYSTAL-1 biliary cohort. KRAS G12C is rare in biliary cancer (roughly 1%), and the registry trial under NCT05722327 is a colorectal-only combination that does not admit biliary patients. A clinical trial remains a reasonable route if an open KRAS G12C basket accepting biliary tumors can be located.

Next steps

1. Confirm KRAS G12C specifically on tumor DNA NGS; other KRAS variants do not qualify for a G12C inhibitor.
2. For off-label biliary use, expect a prior-authorization step; cite the published KRYSTAL-1 biliary cohort (PMID 37099736) and call BMS Medical Information at 1-800-721-5072.
3. Search for a currently recruiting KRAS G12C basket that explicitly accepts biliary tract cancer, since NCT05722327 as registered is colorectal-only and closed.
4. Flag to the trial_screener that the NCT05722327 record does not match the biliary-basket description in the dossier.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05722327		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Bristol Myers Squibb (Mirati Therapeutics)
- **Med info phone:** 1-800-721-5072
- **Product info:** <https://www.krazati.com/>
- **Notes:** 1-800-721-5072 is the BMS Medical Information line. BMS Access Support assists with benefits and prior authorization. No public medical-info email found.

Payer / coverage: No biliary indication exists, so any biliary use is off-label and likely to face prior-authorization hurdles. The published KRYSTAL-1 biliary cohort was small (n=12, ORR about 42%) but supportive. A KRAS G12C inhibitor only applies to G12C; G12D/G12V and other KRAS variants do not qualify.

11. Trastuzumab deruxtecan (T-DXd)

Aliases: trastuzumab deruxtecan, T-DXd, DS-8201, fam-trastuzumab deruxtecan-nxki, Enhertu

Modality: ADC · **Regulatory:** FDA-approved (accelerated, April 2024) tumor-agnostic for previously treated, unresectable or metastatic HER2-positive (IHC 3+) solid tumors; biliary tract cancer was part of the supporting DESTINY-PanTumor02 data · **Guidelines:** NCCN-listed for HER2 IHC 3+ biliary tract cancer under the tumor-agnostic HER2 indication · **Geographic scope:** US; first-line trial NCT06467357 recruiting at multiple US sites · **Verified:** 2026-06-01

Because the HER2 IHC 3+ label is tumor-agnostic, T-DXd is effectively on-label for an IHC 3+ biliary tumor once profiling confirms it; below IHC 3+ it shifts to a genuine off-label call. A first-line randomized trial, DESTINY-BTC01 (NCT06467357), is recruiting across US sites for treatment-naive HER2-expressing patients. ILD risk should weigh in the decision even though no toxicity veto was set.

Next steps

1. Run HER2 IHC with reflex ISH and document the exact IHC score, since 3+ is the on-label threshold and drives expected benefit.
2. If treatment-naive, call the AstraZeneca center at 1-877-240-9479 to check DESTINY-BTC01 (NCT06467357) slots before defaulting to off-label use.
3. For IHC 3+ use, the oncologist can prescribe on-label; for IHC 2+ expect a prior-authorization step.
4. Baseline pulmonary assessment and active ILD/pneumonitis surveillance throughout treatment.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06467357		recruiting	unconfirmed	AstraZeneca Clinical Study Information Center; information.center@astrazeneca.com; 1-877-240-9479

NCT04422309	active not recruiting	unconfirmed	—
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Manufacturer / sponsor contact

- **Company:** AstraZeneca / Daiichi Sankyo
- **Med info phone:** 1-877-240-9479
- **Med info email:** information.center@astrazeneca.com
- **Product info:** <https://www.enhertu.com/>
- **Notes:** 1-877-240-9479 is the AstraZeneca information center (verified via the NCT06467357 registry record).
The tumor-agnostic label means biliary use is on-label for HER2 IHC 3+, off-label below that.

Payer / coverage: The tumor-agnostic HER2 IHC 3+ label makes biliary use on-label at IHC 3+. Use at IHC 2+ falls outside the label and is more likely to draw a prior-authorization fight; the DESTINY-PanTumor02 data showed the IHC 3+ subgroup responded far better (about 61% vs 22% overall). Interstitial lung disease is the defining safety risk and was fatal in three patients on the basket study.

Clinical trial (1)

12. TYRA-200

Aliases: TYRA-200

Modality: small_molecule · **Regulatory:** Investigational; no regulatory approval anywhere · **Geographic scope:** US sites only (UCSF, MGH, Ohio State, MD Anderson) · **Verified:** 2026-06-01

TYRA-200 is a next-generation FGFR2 inhibitor in phase 1, and the only way to get it is enrolling in NCT06160752. It is most relevant if the patient tests FGFR2-positive and later progresses on pemigatinib or futibatinib, since it targets some of the resistance mutations that defeat the approved drugs. No efficacy data have read out yet.

Next steps

1. Confirm an activating FGFR2 alteration first, since eligibility turns on it.
2. Email the central contact at TyraClinicalTrials@tyra.bio or call (619) 728-4805 to check slot availability at the nearest of UCSF, MGH, Ohio State, or MD Anderson.
3. Reserve this option mainly for the post-FGFR-inhibitor resistance setting rather than treatment-naïve FGFR2-positive disease, where approved drugs come first.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06160752		recruiting	unconfirmed	Grace Indyk; TyraClinicalTrials@tyra.bio ; (619) 728-4805

Manufacturer / sponsor contact

- **Company:** Tyra Biosciences
- **Med info phone:**(619) 728-4805
- **Med info email:**TyraClinicalTrials@tyra.bio
- **Notes:** Same contact serves trial inquiries; no separate medical-information line exists for an investigational agent.

Payer / coverage: Investigational; trial sponsor covers study-drug costs. No commercial coverage and no off-label path because the drug is not approved.
